

CLINICAL OPTIMIZATION

Inpatient Workflow and Documentation Optimization

Hypothetical Client Deliverable

Regional Health System | Illustrative Example

Purpose

Demonstrate the type of client-facing work product that could support a clinical optimization engagement. This document is fictional and is intended for website demonstration only.

Prepared for demonstration purposes

July 2026

Executive Summary

The health system is experiencing variation in inpatient workflows, inconsistent documentation practices, and uneven use of the electronic health record across facilities. These issues increase clinician burden, create avoidable rework, and make it harder to sustain reliable care delivery.

18%
Estimated documentation time reduction

26
Priority workflow opportunities

3
Pilot inpatient units

90 Days
Initial implementation horizon

Recommended Direction

Priorit y	Recommendation	What it means
1	Standardize high-volume workflows	Define common future-state workflows for admission, transfer, discharge, medication reconciliation, and interdisciplinary rounds.
2	Reduce documentation burden	Eliminate duplicate fields, clarify documentation ownership, and simplify role-based templates.
3	Strengthen clinical governance	Create a multidisciplinary governance structure to approve standards, prioritize changes, and monitor adoption.
4	Pilot, measure, and scale	Test improvements in selected units, evaluate performance, refine the approach, and scale across the enterprise.

Expected impact: Less clinician rework, more consistent documentation, stronger workflow adoption, improved patient throughput, and clearer accountability for clinical optimization decisions.

1. Clinical Workflow Assessment

The assessment combines stakeholder interviews, workflow observations, EHR usage data, policy review, and frontline validation. Findings are organized around the moments in the inpatient journey where variation creates the greatest operational and clinical impact.

Area	Illustrative current-state finding	Operational consequence	Priority
Admission	Multiple admission documentation paths and inconsistent role ownership	Duplicate entry and delayed completion	High
Medication Reconciliation	Different handoffs between nursing, pharmacy, and providers	Rework and incomplete reconciliation	High
Interdisciplinary Rounds	Unit-specific formats and inconsistent escalation criteria	Delayed decisions and variable discharge planning	High
Discharge	Documentation, education, and follow-up tasks occur in different sequences	Longer discharge cycle time	High
Clinical Documentation	Redundant fields and unclear documentation expectations	Excess time in the EHR	Medium
Governance	Optimization requests lack a consistent intake and approval process	Backlog growth and uneven prioritization	Medium

Opportunity Prioritization Criteria

Clinical impact Patient safety, care quality, and reliability.	Clinician burden Time, clicks, rework, and cognitive load.	Operational impact Throughput, delays, and handoff efficiency.	Feasibility Complexity, dependencies, and time to value.
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2. Standardized Future-State Workflow Designs

The future-state model standardizes core workflow expectations while allowing governed exceptions for specialty-specific clinical needs.

1. ADMIT	2. ASSESS	3. COORDINATE	4. PREPARE	5. DISCHARGE
Complete required admission elements once, with clear role ownership.	Use role-based documentation templates aligned to the plan of care.	Conduct standardized interdisciplinary rounds with escalation criteria.	Begin discharge planning early with visible readiness milestones.	Complete education, reconciliation, follow-up, and final documentation.

Future-State Design Principles

- Document once and reuse information wherever possible.
- Assign clear ownership for every required clinical task.
- Design workflows around the clinician role and point of care.
- Use system controls for high-risk steps; avoid unnecessary hard stops.
- Standardize enterprise-wide while governing justified exceptions.

Example Workflow Standard: Discharge Readiness

Milestone	Accountable role	Evidence of completion
Expected discharge date documented	Attending provider	Visible expected date and disposition
Medication reconciliation initiated	Provider / pharmacy	Open issues identified before discharge day
Patient education completed	Nursing	Education documented using standardized template
Follow-up needs confirmed	Care management	Appointments, referrals, and services documented
Final discharge order and instructions	Attending provider	Order, instructions, and reconciled medication list complete

3. Prioritized Clinical Optimization Roadmap

The roadmap sequences rapid improvements with the governance, technology, and adoption capabilities required to sustain change.

Workstream	0-30 Days	31-60 Days	61-90 Days	Beyond 90 Days
Governance	Form steering group; approve intake criteria	Launch decision cadence	Review pilot results	Scale governance enterprise-wide
Workflow	Validate current-state findings	Design future-state workflows	Pilot in selected units	Scale prioritized workflows
EHR Optimization	Confirm high-burden documentation issues	Configure and unit test changes	Deploy pilot build	Expand approved changes
Training & Adoption	Segment audiences and impacts	Develop role-based materials	Train pilot units and support go-live	Refresh training based on usage
Measurement	Confirm baseline measures	Build pilot dashboard	Monitor outcomes weekly	Transition to operational scorecard

Illustrative Pilot Scope

Medical Unit Admission documentation and interdisciplinary rounds.	Surgical Unit Medication reconciliation and discharge readiness.	Step-Down Unit Handoff standardization and documentation burden.
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4. Role-Based Training and Adoption Materials

Training is designed around the work each audience must perform, supported by manager reinforcement and at-the-elbow assistance during implementation.

Audience	Learning objective	Primary support
Physicians	Use standardized documentation and discharge workflows	Brief workflow demonstrations, tip sheets, peer champions
Nursing	Complete role-based documentation and escalation steps	Scenario-based training, job aids, unit huddles
Care Management	Coordinate discharge readiness and follow-up requirements	Workflow labs, process maps, office hours
Clinical Leaders	Monitor adoption and address local barriers	Leader toolkit, dashboard training, escalation guide
Support Teams	Triage workflow, configuration, and access issues	Support scripts, issue categories, ownership matrix

5. Clinical Governance and Decision-Making Framework

EXECUTIVE SPONSORS
Chief Medical Officer Chief Nursing Officer Chief Information Officer
CLINICAL OPTIMIZATION STEERING COMMITTEE
Clinical operations, informatics, quality, IT, physician, nursing, and ancillary leaders
DESIGN WORKGROUPS
Workflow owners Frontline clinicians Analysts Educators Application specialists

Decision rule: Enterprise standards are the default. Exceptions require a documented clinical rationale, accountable owner, review date, and measurable outcome.

6. KPI Dashboard and Performance-Monitoring Plan

The scorecard combines clinical, operational, experience, and adoption measures so leaders can evaluate whether changes improve care delivery rather than simply increase system usage.

Category	Illustrative KPI	Definition	Review cadence
Clinician Burden	Documentation time per patient day	Median active documentation time by role	Weekly during pilot
Workflow Adoption	Future-state workflow compliance	Percent of eligible encounters following the approved workflow	Weekly during pilot
Throughput	Discharge order-to-departure time	Elapsed time from final order to patient departure	Weekly
Quality	Medication reconciliation completion	Percent completed before discharge	Weekly
Experience	Clinician ease-of-use pulse score	Short survey on usability and workflow fit	Biweekly
Support	Optimization issues by category	Volume, severity, owner, and aging of issues	Daily during go-live

Illustrative 90-Day Success Measures

10-15% Reduction in documentation time	>85% Workflow adoption in pilot units	15 Min Discharge cycle-time improvement	<5 Days Average issue resolution time
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Next Steps

- Confirm executive sponsors and pilot-unit leadership.
- Validate the assessment findings with frontline clinicians.
- Approve the 90-day pilot scope, success measures, and governance cadence.
- Launch workflow design sessions and establish the baseline dashboard.

This document is a hypothetical representation of a clinical optimization deliverable and does not describe an actual client, engagement, or guaranteed outcome.